



Radiology Report – CT Chest (With IV Contrast)

Department: Diagnostic Imaging – CT
Facility: NorthStar Health Network – CT Suite
Exam: CT Chest with IV Contrast
Date of Exam: 2022-02-14
Comparison: None
Radiologist: Network Attending (synthetic signature)

****Indication:**** Chronic lymphadenopathy workup. Evaluate for intrathoracic adenopathy and characterize incidental pulmonary findings.

****Technique:**** CT chest with IV contrast. Thin-section axial imaging with coronal and sagittal reconstructions. Automated exposure control used. No contrast reaction documented.

****Findings – Lungs:**** Multiple small solid nodules are present bilaterally. Representative nodules include: right upper lobe 4 mm; right middle lobe 3 mm; right lower lobe subpleural 4 mm; left lower lobe 3 mm. No cavitation, ground-glass component, or suspicious spiculation is identified. No focal consolidation. Mild emphysematous background change is noted.

****Findings – Airways/Pleura:**** Central airways are patent without endobronchial lesion. No pleural effusion or pneumothorax. No pleural-based mass.

****Findings – Mediastinum/Hila:**** No enlarged mediastinal or hilar lymph nodes by size criteria. No abnormal mediastinal soft tissue mass. Heart size normal. No pericardial effusion. Great vessels unremarkable within non-angiographic limits.

****Findings – Upper Abdomen (limited):**** Visualized liver, spleen, adrenals, and upper kidneys demonstrate no acute abnormality within the limited field.

****Impression:**** (1) Bilateral subcentimeter pulmonary nodules up to 4 mm; appearance favors benign etiology in low-risk patient. (2) No intrathoracic lymphadenopathy. (3) Mild emphysematous change—correlate with exposure history and consider spirometry if clinically indicated.

****Recommendation:**** Optional surveillance CT in 6–12 months depending on risk factors and shared decision-making; if prior imaging exists, comparison for stability is valuable.



Radiology Addendum – Guideline Context

For solid pulmonary nodules <6 mm in low-risk adults, many guideline frameworks support no routine follow-up; however, clinical judgment may prompt interval surveillance when there is uncertainty regarding risk factors, when multiple nodules are present, or when the broader clinical context includes unexplained systemic findings. Shared decision-making is recommended, balancing radiation exposure, patient anxiety, and likelihood of clinically meaningful change.